

Interstitial cystitis (IC), also called bladder pain syndrome, is a long-term condition. It causes **pain, pressure or discomfort in the bladder or pelvis**, along with a frequent or urgent need to urinate. It is **not** caused by an infection. The pain often builds as the bladder fills and eases after you urinate. There is no single cure, but it can be managed well.

About This Condition

IC is not an infection and you cannot catch it or pass it on. Symptoms differ from person to person. They often come in **flares** (worse spells) with calmer times in between. Common features:

- Pain, pressure or discomfort in the bladder or pelvis
- A frequent or urgent need to urinate, day and night
- Pain that **gets worse as the bladder fills** and eases after urinating
- Flares that may follow some foods, stress or your menstrual cycle

What Causes It

The exact cause is not known. It likely involves an irritated bladder lining, nerves that are too sensitive, and often **tight pelvic floor muscles**. Because there are several causes, treatment usually mixes a few approaches.

Learn the Terms

IC

Interstitial cystitis, also called bladder pain syndrome. Bladder pain and urgency that is not from infection.

Flare

A spell of worse symptoms, often set off by food or stress.

Pelvic floor therapy

Hands-on physical therapy to relax tight pelvic muscles.

Bladder instillation

Medicine placed into the bladder through a thin tube.

Hunner lesion

A red, sore patch on the bladder wall seen in some people. It can be treated directly.

Cystoscopy

A look inside the bladder with a thin camera.

Will it ever go away? IC is usually long-term. But most people get real relief by mixing treatments and learning their triggers. The goal is fewer, milder flares and a normal daily life. Finding your mix often takes some trial and error, so give it time.

How It's Diagnosed

There is no single test. Your team makes the diagnosis from your symptoms, after ruling out other causes:

- A **urine test** to rule out infection
- Questions about your symptoms, and a **bladder diary**
- A **cystoscopy** for some people, such as when a Hunner lesion is possible

How It's Treated

Treatments are often combined. They do not have to be tried in a set order.

- **Self-care:** learn your **food triggers**, use heat or cold on the pelvis, manage stress, and treat constipation.
- **Pelvic floor physical therapy** to relax tight muscles. Avoid Kegel squeezing exercises, which can make IC worse.
- **Medicines:** pills, or medicine placed into the bladder. One pill (pentosan) can harm the eyes, so it needs eye exams.
- **Other options:** treating a Hunner lesion, Botox in the bladder, or nerve stimulation.

Living With IC

- Common triggers include **coffee, alcohol, citrus, tomatoes, fizzy drinks and spicy foods**. Remove one, then add it back, to learn yours.
- Have a **flare plan**, such as rest and heat.
- Long-term antibiotics and steroid pills do not help IC.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, chills, or pain in your side or back
- New **blood in the urine**
- A flare you cannot control, or vision changes while on pentosan

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You cannot reach the office and are worried

Three Things to Remember

1. IC is bladder pain with urgency that is **not** an infection. It is long-term but manageable.
2. Most people do best with a mix of self-care, pelvic floor therapy and medicines.
3. Learn your triggers and keep a flare plan.